

Supplement

Supplementary slides to Clarke *et al* presentation: *Final pre-specified overall survival in PROpel: abiraterone and olaparib versus abiraterone and placebo as first-line therapy for metastatic castration-resistant prostate cancer*

Presented at the 2023 Annual Meeting of ASCO-GU, held on February 16–18, 2023 in San Francisco, CA, USA. Abstract Number: LBA 16

PROpel: patient disposition (DCO3)

	Abiraterone + olaparib (N=399)	Abiraterone + placebo (N=397)	Total (N=796)
Patients enrolled			1103
Patients randomized	399 (100)	397 (100)	796 (100)
Patients who were not randomized			307
Screen failure			284
Patient decision			20
Incorrect enrollment			2
Other			1
Full analysis set	399 (100)	397 (100)	796 (100)
Patients who received treatment	398 (99.7)	396 (99.7)	794 (99.7)
Patients who did not receive treatment	1 (0.3)	1 (0.3)	2 (0.3)
Failure to meet randomization criteria	1 (0.3)	0	1 (0.1)
Screen failure	0	1 (0.3)	1 (0.1)
Patients ongoing study at DCO3	210 (52.6)	178 (44.8)	388 (48.7)
Patients who terminated study at DCO3	189 (47.4)	219 (55.2)	408 (51.3)
Patients ongoing treatment at DCO3	110 (27.6)	79 (19.9)	189 (23.8)
Patients ongoing both olaparib/placebo and abiraterone	103 (25.9)	77 (19.4)	180 (22.7)
Patients who discontinued treatment at DCO3	288 (72.4)	317 (80.1)	605 (76.2)

Percentages are calculated from number of patients who received treatment. DCO3: 12 October 2022. DCO3, third data cut-off. Supplementary slides to Clarke *et al* presentation: *Final overall survival in PROpel: abiraterone and olaparib versus abiraterone and placebo as first-line therapy for metastatic castration-resistant prostate cancer* presented at the 2023 Annual Meeting of ASCO-GU, held on February 16–18, 2023 in San Francisco, CA, USA. Abstract Number: LBA 16

PROpel: pre-planned analyses

**Primary rPFS analysis
(DCO1)¹
30 July 2021**

**Interim OS analysis
(DCO2)
14 March 2022**

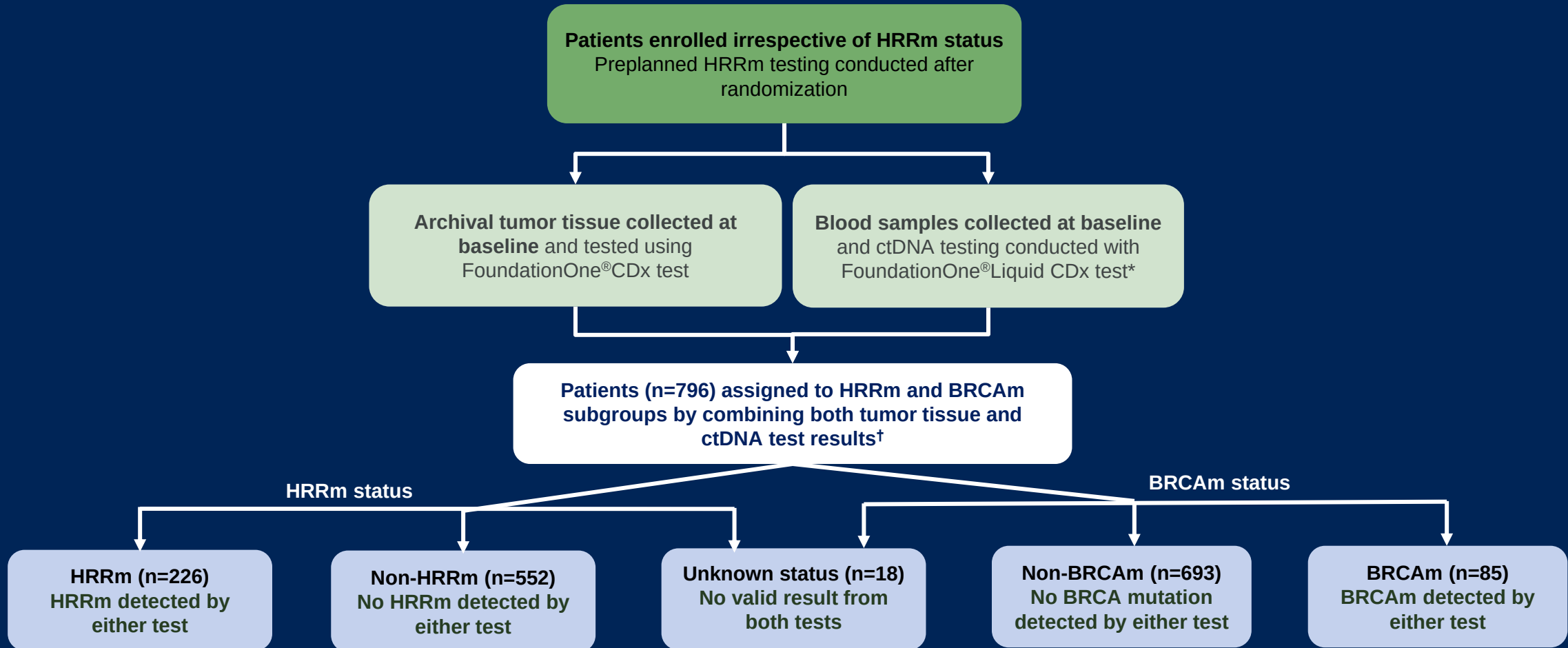
**Final pre-specified
OS analysis
(DCO3)
12 Oct 2022**

EU approval (EC decision) 16 Dec 2022: olaparib is indicated in combination with abiraterone and prednisone or prednisolone for the treatment of adult patients with mCRPC in whom chemotherapy is not clinically indicated

DCO, data cut-off; EC, European Commission; EU, European Union; mCRPC, metastatic castration-resistant prostate cancer; OS, overall survival; rPFS, radiographic progression-free survival. Supplementary slides to Clarke *et al* presentation: *Final overall survival in PROpel: abiraterone and olaparib versus abiraterone and placebo as first-line therapy for metastatic castration-resistant prostate cancer* presented at the 2023 Annual Meeting of ASCO-GU, held on February 16–18, 2023 in San Francisco, CA, USA. Abstract Number: LBA 16

1. Clarke N *et al*. *NEJM Evidence* 2022;1(9).

PROpel: HRRm and BRCAm assignment



*For the ctDNA-based test, ctDNA tumor fraction (%) could be estimated for 92% of samples (673/734) that yielded an NGS result, and the median ctDNA fraction estimate was 10% (interquartile range 1–26%).

†Aggregate analyses were *post hoc*. BRCAm, *BRCA1* and/or *BRCA2* mutation; ctDNA, circulating tumor DNA; HRRm, homologous recombination repair mutation.

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PROpel: Summary of rPFS and OS results

A continued rPFS and OS benefit was observed across the data cut-offs

Primary
endpoint

rPFS	Primary analysis (DCO1)		Interim OS analysis (DCO2)		Final pre-specified OS analysis (DCO3)*	
	Abiraterone + olaparib (n=399)	Abiraterone + placebo (n=397)	Abiraterone + olaparib (n=399)	Abiraterone + placebo (n=397)	Abiraterone + olaparib (n=399)	Abiraterone + placebo (n=397)
Events, n (%)	168 (42.1)	226 (56.9)	199 (49.9)	258 (65.0)	219 (54.9)	277 (69.8)
Median, months	24.8	16.6	25.0	16.4	25.0	16.5
HR (95% CI)	0.66 (0.54–0.81)		0.67 (0.56–0.81)		0.68 (0.57–0.81)	
OS						
Events, n (%)	107 (26.8)	121 (30.5)	148 (37.1)	171 (43.1)	176 (44.1)	205 (51.6)
Median, months	NR	NR	NR	NR	42.1	34.7
HR (95% CI)	0.86 (0.66–1.12)		0.83 (0.66–1.03)		0.81 (0.67–1.00)	

Secondary
endpoint

DCO1: 30 July 2021; DCO2: 14 March 2022; DCO3: 12 October 2022.

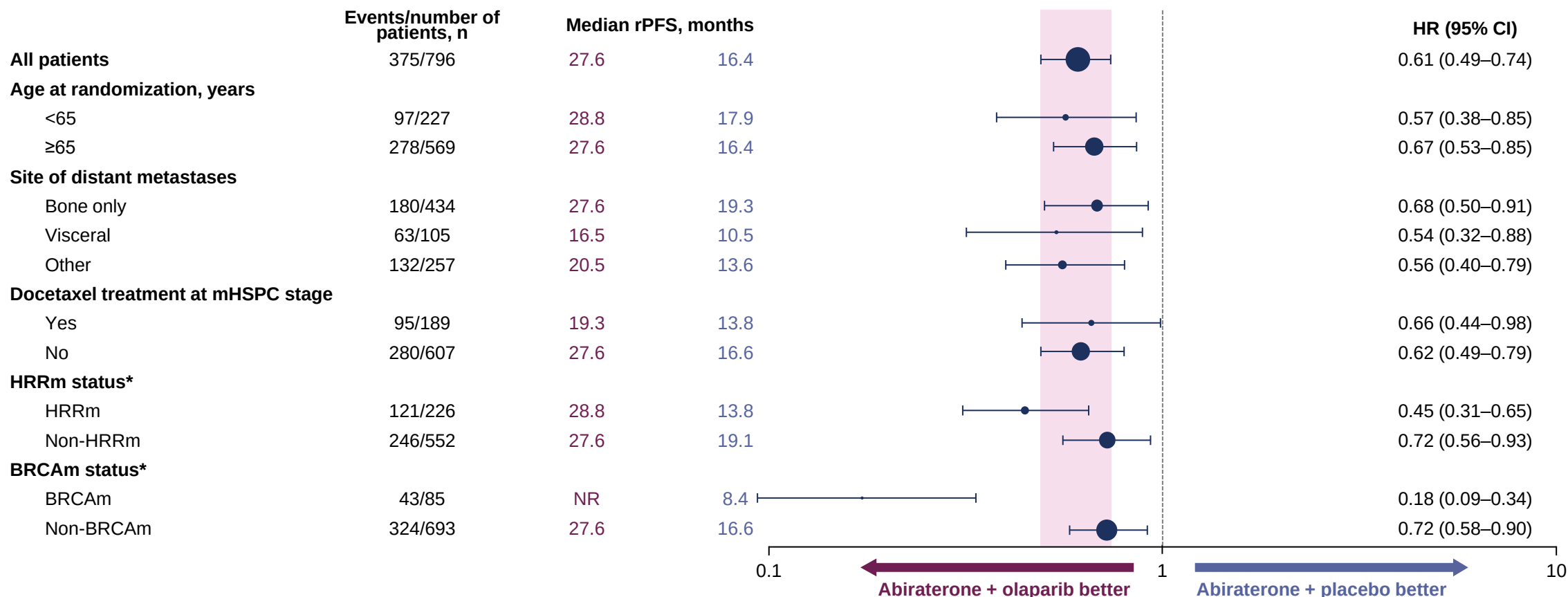
*OS was the endpoint specified for analysis at DCO3. CI, confidence interval; HR, hazard ratio; NR, not reached.

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PROpel: rPFS in subgroups (DCO1; BICR)¹

Consistent with investigator-assessed results



DCO1: 30 July 2021.

*The preplanned tumor tissue and plasma ctDNA testing was conducted after randomization and before primary analysis. Results from tumor tissue and plasma ctDNA were combined to determine patients HRRm status. Results shown are by BICR. BICR, blinded independent central review; mHSPC, metastatic hormone-sensitive prostate cancer.

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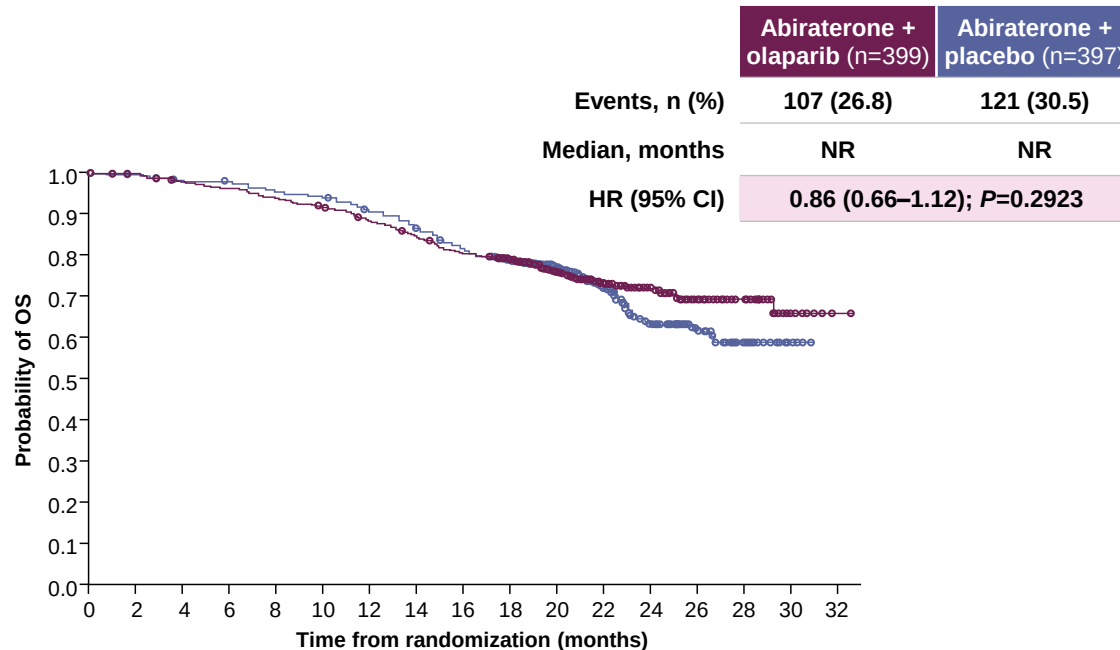
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1. Clarke N *et al*. *NEJM Evidence* Clarke N *et al*. *NEJM Evidence* 2022;1(9). Copyright © 2022 Massachusetts Medical Society. Reprinted with permission from Massachusetts Medical society.

PROpel: OS at DCO1 and DCO2

A trend towards OS benefit with abiraterone + olaparib versus abiraterone + placebo was observed

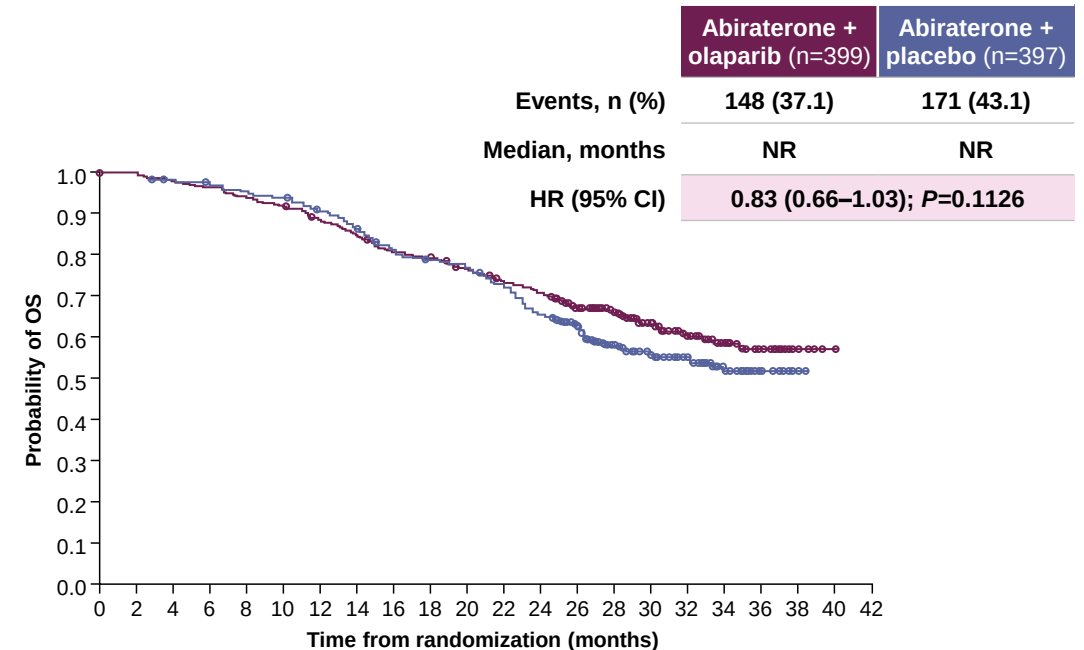
DCO1 (28.6% maturity)¹



Number of patients at risk:

	399	398	391	385	374	364	349	333	316	290	231	159	116	73	37	11	1
Abiraterone + olaparib	399	398	391	385	374	364	349	333	316	290	231	159	116	73	37	11	1
Abiraterone + placebo	397	392	385	381	374	368	353	335	314	286	223	151	104	63	22	6	0

DCO2 (40.1% maturity)²



Number of patients at risk:

	399	398	391	385	374	365	350	335	318	313	298	284	274	232	188	135	93	53	23	5	1	0
Abiraterone + olaparib	399	398	391	385	374	365	350	335	318	313	298	284	274	232	188	135	93	53	23	5	1	0
Abiraterone + placebo	397	395	388	383	376	370	355	337	316	304	300	280	253	211	154	106	81	46	13	1	0	0

DCO1: 30 July 2021; DCO2: 14 March 2022. Median (range) duration of follow-up for censored patients at DCO1 was 22.2 months (>0.1–32.6) in the abiraterone + olaparib arm and 21.8 months (0.1–30.9) in the abiraterone + placebo arm. Median (range) duration of follow-up for censored patients at DCO2 was 30.0 months (>0.1–40.0) in the abiraterone + olaparib arm and 29.4 months (2.9–38.3) in the abiraterone + placebo arm.

1. Clarke N *et al. NEJM Evidence* 2022;1(9). Copyright © 2022 Massachusetts Medical Society. Reprinted with permission from Massachusetts Medical society. 2. Saad *et al. Annals of Oncology* 2022;33(suppl_7):S616–52.

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PROpel: overall safety profile

No new safety signals with longer treatment duration and follow-up

	Abiraterone + olaparib (n=398)			Abiraterone + placebo (n=396)		
N (%)	DCO1	DCO2	DCO3	DCO1	DCO2	DCO3
Any AE	387 (97.2)	389 (97.7)	389 (97.7)	376 (94.9)	378 (95.5)	380 (96.0)
Any AE CTCAE Grade ≥3	188 (47.2)	210 (52.8)	222 (55.8)	152 (38.4)	160 (40.4)	171 (43.2)
Death due to an AE	16 (4.0)	23 (5.8)	26 (6.5)	17 (4.3)	18 (4.5)	20 (5.1)
Any AE leading to:						
Dose interruption of olaparib or placebo	178 (44.7)	190 (47.7)	195 (49.0)	100 (25.3)	108 (27.3)	112 (28.3)
Dose reduction of olaparib or placebo	80 (20.1)	85 (21.4)	90 (22.6)	22 (5.6)	22 (5.6)	24 (6.1)
Discontinuation of olaparib or placebo	55 (13.8)	63 (15.8)	69 (17.3)	31 (7.8)	32 (8.1)	34 (8.6)
Discontinuation of abiraterone	34 (8.5)	41 (10.3)	45 (11.3)	35 (8.8)	35 (8.8)	37 (9.3)

More details on AEs of special interest for olaparib

- New primary malignancies: 18 cases in the abiraterone + olaparib arm and 14 cases in the abiraterone + placebo arm
- Pneumonitis: 5 cases in the abiraterone + olaparib arm and 3 cases in the abiraterone + placebo arm

DCO1: 30 July 2021; DCO2: 14 March 2022; DCO3: 12 October 2022. Patients ongoing treatment at DCO3: 110 for abiraterone + olaparib; 79 for abiraterone + placebo.

At DCO3, median total treatment duration of olaparib was 18.5 months, placebo was 15.7 months, abiraterone in the abiraterone + olaparib arm was 20.1 months and in the abiraterone + placebo arm was 15.7 months.

AE, adverse event; CTCAE, Common Terminology Criteria for Adverse Events v4.03.

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PROpel: cardiac failure SMQ AEs

	Abiraterone + olaparib (n=398)			Abiraterone + placebo (n=396)		
n (%)	DCO1	DCO2	DCO3	DCO1	DCO2	DCO3
Cardiac failure SMQ	6 (1.5)	7 (1.8)	7 (1.8)	5 (1.3)	7 (1.8)	7 (1.8)
Acute pulmonary edema	0	0	0	1 (0.3)	2 (0.5)	2 (0.5)
Cardiac failure	2 (0.5)	3 (0.8)	3 (0.8)	2 (0.5)	2 (0.5)	2 (0.5)
Cardiac failure acute	1 (0.3)	1 (0.3)	1 (0.3)	0	0	0
Cardiac failure chronic	0	0	0	1 (0.3)	1 (0.3)	1 (0.3)
Cardiac failure congestive	1 (0.3)	1 (0.3)	1 (0.3)	1 (0.3)	2 (0.5)	2 (0.5)
Ejection fraction decreased	1 (0.3)	1 (0.3)	1 (0.3)	0	0	0
Left ventricular failure	2 (0.5)	2 (0.5)	2 (0.5)	0	0	0
Pulmonary edema	0	0	0	2 (0.5)	2 (0.5)	2 (0.5)

DCO1: 30 July 2021; DCO2: 14 March 2022; DCO3: 12 October 2022.

Patients with multiple events in the same category are counted only once in that category. Patients with events in more than one category are counted once in each of those categories. DCO2 results include AEs reported at DCO1 and DCO2. DCO3 results include AEs reported at DCO1, DCO2 and DCO3.

SMQ, standardized medDRA query.

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PROpel: embolic and thrombotic events, arterial SMQ AEs

	Abiraterone + olaparib (n=398)			Abiraterone + placebo (n=396)		
n (%)	DCO1	DCO2	DCO3	DCO1	DCO2	DCO3
Embolic and thrombotic events, arterial SMQ	8 (2.0)	10 (2.5)	10 (2.5)	10 (2.5)	13 (3.3)	14 (3.5)
Acute coronary syndrome	0	0	0	0	0	1 (0.3)
Acute myocardial infarction	3 (0.8)	4 (1.0)	4 (1.0)	3 (0.8)	3 (0.8)	3 (0.8)
Ischemic stroke	1 (0.3)	1 (0.3)	1 (0.3)	4 (1.0)	4 (1.0)	4 (1.0)
Myocardial infarction	1 (0.3)	1 (0.3)	1 (0.3)	1 (0.3)	4 (1.0)	4 (1.0)
Peripheral arterial occlusive disease	2 (0.5)	3 (0.8)	3 (0.8)	0	0	0
Peripheral embolism	0	0	0	1 (0.3)	1 (0.3)	1 (0.3)
Transient ischemic attack	1 (0.3)	1 (0.3)	1 (0.3)	1 (0.3)	1 (0.3)	1 (0.3)

DCO1: 30 July 2021; DCO2: 14 March 2022; DCO3: 12 October 2022.

Patients with multiple events in the same category are counted only once in that category. Patients with events in more than one category are counted once in each of those categories. DCO2 results include AEs reported at DCO1 and DCO2. DCO3 results include AEs reported at DCO1, DCO2 and DCO3.

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PROpel: embolic and thrombotic events, venous SMQ

	Abiraterone + olaparib (n=398)			Abiraterone + placebo (n=396)		
n (%)	DCO1	DCO2	DCO3	DCO1	DCO2	DCO3
Embolic and thrombotic events, venous SMQ	29 (7.3)	32 (8.0)	34 (8.5)	13 (3.3)	13 (3.3)	16 (4.0)
Deep vein thrombosis	7 (1.8)	9 (2.3)	10 (2.5)	3 (0.8)	3 (0.8)	3 (0.8)
Portal vein thrombosis	0	0	0	1 (0.3)	1 (0.3)	1 (0.3)
Pulmonary embolism	26 (6.5)	28 (7.0)	29 (7.3)	7 (1.8)	7 (1.8)	9 (2.3)
Retinal vein occlusion	0	0	0	0	0	1 (0.3)
Superficial vein thrombosis	0	0	0	2 (0.5)	2 (0.5)	2 (0.5)

- The majority of pulmonary embolism events were asymptomatic and incidental on imaging

DCO1: 30 July 2021; DCO2: 14 March 2022; DCO3: 12 October 2022.

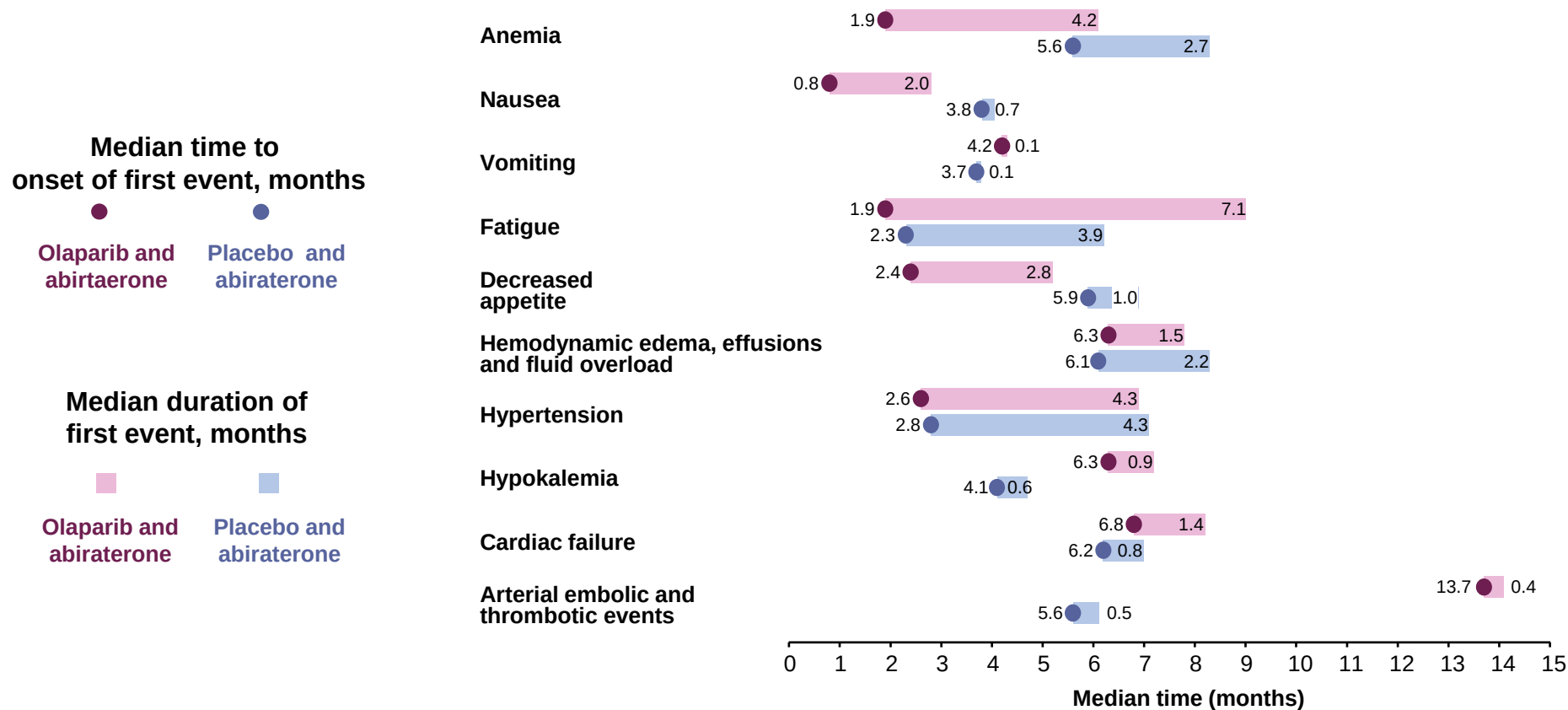
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PROpel: timings of AEs at DCO1

Most of the known abiraterone and olaparib (all grades) AEs occurred within 6 months, and the addition of olaparib to abiraterone did not prolong the duration of the first event*



Median total duration of exposure:

Abiraterone and olaparib arm

Abiraterone 18.2 months

Olaparib 17.5 months

Abiraterone and placebo:

Abiraterone 15.7 months

Placebo: 15.7 months

*AEs reported based on being commonly associated with either olaparib or abiraterone. Supplementary slides to Clarke *et al* presentation: *Final overall survival in PROpel: abiraterone and olaparib versus abiraterone and placebo as first-line therapy for metastatic castration-resistant prostate cancer* presented at the 2023 Annual Meeting of ASCO-GU, held on February 16–18, 2023 in San Francisco, CA, USA. Abstract Number: LBA 16